

NURSING WITH LOLO

Nursing Foundations

ADPIE, safety, communication, delegation, mobility, and essential bedside priorities.

Simple explanations - nursing priorities - memory cues - original practice - rationales

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Educational study aid - verify current instructor and facility requirements

What you will be able to do

Use these objectives to guide active recall before and after the lesson.

Learning objectives

- Connect ADPIE with clinical-judgment steps.
- Separate relevant cues from background information.
- Choose and evaluate a patient-centered priority action.
- Recognize patient, medication, and environmental fall risks.
- Select individualized fall-prevention actions.
- Respond safely after a fall or near miss.

How to use this guide

- Read the simple overview once.
- Close the guide and explain the priority in your own words.
- Answer the practice items before opening the rationale.
- Return to the lowest-confidence section tomorrow.

The concept, made simple

Plain language first; precise nursing language follows.

Core explanations

- ADPIE means assess, diagnose or analyze, plan, implement, and evaluate. The steps guide thinking, but nurses often move back and forth as the patient changes.
- Clinical judgment begins by noticing cues, deciding what they may mean, ranking likely concerns, responding safely, and checking whether the response worked.
- The first goal is not to name every possible problem. It is to identify the most urgent, supported concern while continuing to collect useful information.
- Fall risk is not a label that stays the same all day. Illness, new symptoms, medications, toileting needs, and unfamiliar surroundings can change risk quickly.
- The best prevention plan matches the patient. It combines clear communication, a safer environment, appropriate assistance, and frequent reassessment.
- A fall-prevention tool is only helpful when the team uses it consistently and the patient understands how to ask for help.

Vocabulary move

- Name the body system or safety process.
- Connect the cue to the risk.
- State what the nurse should notice and do first.

Assess before you act

A focused assessment organizes the data that make the next action safe.

What to notice

- Begin with immediate safety and a rapid overview before gathering details.
- Compare current findings with baseline, trends, expected recovery, and patient concerns.
- Validate surprising data by checking the patient, technique, equipment, and record.
- Cluster related subjective and objective cues instead of interpreting one value alone.
- Use the approved fall-risk process on admission and when condition or treatment changes.
- Assess gait, strength, cognition, vision, dizziness, elimination needs, and prior falls.
- Observe the patient actually transferring or walking when safe; do not rely only on self-report.
- Review the room, footwear, bed or chair setup, alarms, lines, and mobility equipment.

Trend, do not snapshot

- Compare with baseline.
- Cluster related cues.
- Validate unexpected values.
- Escalate time-sensitive changes.

Priority actions and safety boundaries

The safest answer protects the client while staying inside role, order, and policy.

Priority actions

- Address an immediate threat to life or safety first.
- If no immediate threat exists, assess before acting and validate uncertain cues.
- Escalate deterioration promptly through the designated chain of communication.
- Evaluate the response and revise the plan rather than assuming an action worked.
- If a patient begins to fall, protect the head and guide the descent if safely possible; do not try to hold full body weight upright.
- After a fall, stay with the patient, assess for immediate threats and injury, and summon help.
- Follow facility policy before moving the patient after a fall.
- Report, document, and reassess the prevention plan after a fall or near miss.

Safety warnings

- Do not delay urgent help while trying to complete a full assessment.
- Do not treat a monitor or chart value without checking the patient.
- Use current facility escalation criteria and instructor guidance in clinical settings.
- Raised side rails, alarms, or signs do not replace observation and individualized care.
- Restrictive measures can cause harm and must follow law, policy, assessment, and authorized orders.
- Never leave a high-risk patient unsupported during a transfer that requires assistance.
- Never alter, backdate, or delete a record outside the approved correction process.

Memory that transfers

A memory aid is useful only when it leads back to clinical reasoning.

Memory hooks

- ADPIE: Assess, Decide, Plan, Intervene, Evaluate.
- Notice -> Meaning -> Priority -> Action -> Response.
- When the picture changes, circle back to assessment.
- SAFE: Scan risk, Arrange the room, Furnish help, Evaluate again.
- Risk changes when the patient, treatment, or environment changes.
- After a fall: stay, assess, call, then follow policy.

Exam reset

- What can harm the client first?
- What data are missing?
- Is the client stable or unstable?
- Which action is least restrictive and within scope?

Clinical judgment in motion

A patient who was speaking comfortably becomes restless and says breathing suddenly feels harder.

Notice

- New restlessness
- Sudden breathing complaint
- Change from the recent baseline

Interpret

- Possible acute oxygenation or ventilation problem
- Immediate bedside assessment is needed

Respond

- Stay with the patient and assess airway, breathing, circulation, and current vital signs.
- Use the facility escalation pathway while providing in-scope supportive care.
- Reassess continuously and communicate the change and response clearly.

Reflect

- The nurse used the acute change to reprioritize, acted on immediate risk, and evaluated continuously.

Practice before the rationale

Choose the best answer using the cue-risk-action sequence.

Question 1

A previously comfortable patient suddenly reports difficulty breathing. What should the nurse do first?

- A. Perform a rapid bedside airway and breathing assessment
- B. Finish routine documentation
- C. Ask dietary services to hold the tray
- D. Wait for the next scheduled vital signs

Question 2

Which actions help the nurse validate an unexpected assessment finding? Select all that apply.

- A. Recheck the patient
- B. Review the measurement technique
- C. Inspect the equipment
- D. Compare with trends and symptoms
- E. Delete the value because it seems unlikely

Answers, rationales, and printable review

Question 1: A

A sudden breathing change may be unstable, so rapid assessment and timely escalation take priority.

Exam clue: The word "suddenly" signals a new priority cue.

Question 2: A, B, C, D

Validation checks the patient, method, equipment, and context while preserving the original record.

Exam clue: Choose actions that verify rather than dismiss data.

Five-point review

- Use ADPIE as a flexible cycle rather than a rigid checklist.
- Prioritize supported cues, urgent threats, and changes from baseline.
- An intervention is incomplete until the patient's response is evaluated.
- Fall risk is dynamic and must be reassessed after relevant change.
- Individualized assistance, environment, communication, and toileting plans work together.

References

National Council of State Boards of Nursing: 2026 NCLEX-PN Test Plan - <https://www.ncsbn.org/publications/2026-nclex-pn-test-plan>

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Agency for Healthcare Research and Quality: Pressure Injury Prevention in Hospitals Training Program - <https://www.ahrq.gov/patient-safety/settings/hospital/resource/pressure-injury/index.html>

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